

إسم المريض رابعياً : ساميه محمود غمرى على

المحافظة : الجيزة الحي / المدينة : الهرم

العنوان : 0

تاريخ تسجيل الإستمارة : 30-09-2026

رقم البطاقة : 27307091701308 رقم التلفون : 01221394003

العمر (تاريخ الميلاد): 09-07-1973 المهنة: ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام

----- التعليق على التشخيص :

الاسم: د/محمد كيلاني الوظيفة: _____
 الاسم: د/ندى حسن الوظيفة: _____
 الاسم: د/نرمين رمضان الوظيفة: _____
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: ساميه محمود غمرى علي National ID: 27307091701308	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مزيد من الفصحى والتعاليق والاشعار
والاجابة





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



04/28/2026

MDT FORM

Name:	Samia Mahmoud // Ali	MRN:	1441
Age:	53 Y	ID:	27307091701308
Menopausal Sta	prem	Phone:	01221394003
NRS SCORE:	0	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

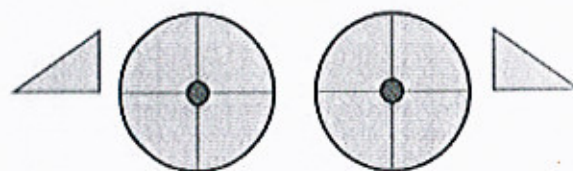
BX: R L
ER: PR: HER2: KI67:

Sonomammography:**Mets Workup:****History:**

PREMENOPAUSAL LEFT BREAST CANCER CT4B N2 MO ER8/8 PR 5/8 HER2 3+
REC NACT TCHP ,ON 21/12/2025 YPT4B N0/17 MDT FOR TDM1,PORT,TAM
THEN SHIFT pt is on kadcyla and tamoxifen mdt to shift to ai as pt become
postmenop and pt is high risk and tamoxifen is not available e2 10 fsh 32

Recommendations:**MDT Outcome & Plan**

TO continue femara up to 7 years

**T N M****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
dr hanan ezzat					

Name: ساميه محمود غامري علي			
MR. : 1441			
Detailed Medical Report			
			السيد
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
نشرف بأن نوضح فيما يلي تقريراً عن حالته			
		تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient was diagnosed with premenopausal left breast cancer, clinical stage T4b N2, estrogen receptor (ER) 8/8 positive, progesterone receptor (PR) 5/8 positive, HER2 3+ positive. Neoadjuvant chemotherapy (NACT) with the TCHP regimen was administered, and on 21/12/2025 the patient was restaged as ypT4b N0/17. Multidisciplinary team (MDT) discussion recommended subsequent therapy with T-DM1, port placement, and tamoxifen, with a planned shift in treatment thereafter. Laboratory studies were reviewed and deemed adequate. The plan was to proceed with cycle 2 (C2) of Kadcyla (ado-trastuzumab emtansine) combined with tamoxifen, with an echocardiogram scheduled before the next visit. The patient received cycle 3 (C3) of Kadcyla plus tamoxifen on the day of the visit; an echocardiogram performed on 10/3 showed a left ventricular ejection fraction of 65%, and laboratory results were accepted. On examination, the patient was noted to be postmenopausal and high-risk; the plan was to continue cycle 4 (C4) of Kadcyla 260 mg and tamoxifen, with MDT discussion to shift to an aromatase inhibitor. The patient's implanted port was removed on 28/2/2026. The MDT meeting in April 2026 recommended continuation of letrozole (Femara) 2.5 mg daily for a total duration of seven years, with supportive care for three months. On 12/7, the patient was evaluated for cycle 7 (C7) of Kadcyla 260 mg and continued Femara; the patient reported epistaxis, and appropriate measures were taken. On examination on 19/08/2026, the patient was evaluated for cycle 8 (C8) of Kadcyla 260 mg, continued Femara, received calcium and vitamin D supplementation, and was prescribed a proton pump inhibitor. The patient has now completed eight cycles of Kadcyla.			
تحريراً في ,			
		التاريخ : 19-09-2026	الوقت :
		dina makram	الطبيب المعالج :
يعتمد ,			

Patient Name : الأستاذ / سامية محمود عمرى على	Request No. : 13009700
Patient No. : 27307091701308	Request Date : 26/04/2025
Path No. : 1479/25	Verification Date : 08/05/2025
Sex : Female	Financial Category : اورام دار السلام - مياذر
Age : 51 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 26/4/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal DCIS (comedo type).
- * Angioinvasion: Undetected.
- * Perineural invasion: Undetected.
- * Microcalcifications: Undetected.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 5% of tumor cells (score 5/8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

Verified by :

Dr.Toka Medhat

ب. ج. فتح م. م.

الحمد لله



Paent Name : الأستاذة / سامية محمود شمري على	Request No. : 13009700
Patient No. : 27307091701308	Request Date : 26/04/2025
Patient No. : 1479/25	Verification Date : 08/05/2025
Sex : Female	Financial Category : مبادئ - اورام دار السلام
Age : 51 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.



* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ghonim.

Dr. Samia Ali.

Verified by :

Dr. Toka Medhat



PATIENT NAME: SAMIA MAHMOUD Ghamry ALI

GENDER: Female

DATE : 15-May-2025 09:47 AM

PATIENT ID: 10125263324

AGE: 51 y

ORDER: 3379473

WHOLE BODY FDG PET-CT EXAMINATION

Clinical background:

- 51-year-old female patient.
- Presented with left breast pathologically proven IDC grade I.
- Weight = 86 Kg.
- BL glucose level = 111 mg/dl.

Reason of study:

- Initial staging study.

Radiopharmaceutical:

- F-18-FDG 5.8 mCi injected.

Technique:

- Approximately one hour after IV injection of the radiopharmaceutical, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- Reference hepatic activity (SUV max)~ 2.24.

Findings:

Reviewing PET, CT and fused PET-CT images:-

I- Breasts and axillary regions:

- Left breast UOQ metabolically active (FDG avid) soft tissue mass is seen infiltrating the overlying skin, measuring about 7.8 x 12.2 cm with SUV max~19.58.
- Multiple FDG avid pathologically enlarged left axillary lymph nodes (levels I, II and III), the largest measures about 4.2 x 5 cm with SUV max~16.55 at level I.

PATIENT: SAMIA MAHMOUD Ghamry ALI, 10125263324, Female, 51 y

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- Few low grade metabolically active left internal mammary lymph nodes, the largest measuring about 0.8 cm with SUV max about 1.31.
- Few prominent low grade metabolically active left supraclavicular lymph nodes, the largest is measuring about 0.8 x 1.5 cm with SUV max about 1.24.
- No metabolically active (FDG avid) contralateral breast masses or axillary lymph nodes.

II. Head and neck:

- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aero-digestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and ary-epiglottic folds. No glottic, supra- or infraglottic masses.
- Intact laryngeal skeleton.
- No FDG avid thyroid nodules.

III. CHEST:

- No metabolically active (FDG avid) pulmonary nodules.
- Non FDG avid right upper lung lobe atelectatic bands likely of old granulomatous disease.
- Few low grade metabolically active mediastinal (right para tracheal, prevascular, retrocaval and subcarinal) lymph nodes, the largest at subcarinal group measuring about 1.3 x 1.7 cm with SUVmax about 2.87.
- No metabolically active pleural effusion or masses.
- Average cardiac size. No pericardial effusion detected.

IV. ABDOMEN AND PELVIS:

- The liver is enlarged with homogenous low CT texture (fatty) with segment IV Isometabolic area of relative hyperdensity mostly representing focall fatty sparing area. No definite CT evidence of metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal appearance of the pancreas, spleen, both kidneys and both suprarenals (on non contrast CT basis) with No FDG avid focal lesions.
- No metabolically active abdominal or pelvic lymphadenopathy.
- Under filling of the urinary bladder with no filling defects or diverticular outpouching.

PATIENT: SAMIA MAHMOUD Ghamry ALI, 10125263324, Female, 51 y

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مركز رؤية الهرم
شماره ١٠١٢٥٢٦٣٣٢٤ شارع الهرم بجوار داري الجيزة

مستشفى رؤية الهرم
شماره ١٠١٢٥٢٦٣٣٢٤ شارع الهرم بجوار داري الجيزة



Patient Name :	الأستاذة / سامية محمود غمري على	Request No. :	13004076
Patient No. :	27307091701308	Request Date :	21/12/2025
Path No. :	3924/25	Verification Date :	01/01/2026
Sex :	Female	Financial Category :	جوستاف روسي - نفقة الدولة
Age :	52 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature of specimen: Left modified radical mastectomy & Extra-axillary lymph nodes.

Gross Examination:

- * Left modified radical mastectomy specimen measured 28x23x5 cm, with covering skin ellipse measured 24x22 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site measured 8x7x3 cm, infiltrating & ulcerating the overlying skin and was 3 cm away from the least deep margin.
- * Axilla: Axillary fat measured 8x7x2 cm, entangling 14 lymph nodes, the largest was 1.5x1x0.5 cm.
- * Extra-axillary lymph nodes: Fibrofatty tissue piece measured 4x3x2 cm, entangling 3 lymph nodes, the largest was 1.5x1x0.5 cm.
- * Total lymph nodes count: 17.
- 15 Blocks:
- 1:8- TBS., 9- N&A., 10- Deep margin., 11:14- ALNs., 15- Extra-axillary fat.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic examination:

- * Histological type: Residual viable invasive duct carcinoma (Actual tumour size 3x3 cm) (Cellularity 70%).
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Tumor ulcerating overlying skin.
- * Ductal carcinoma in situ: Minimal (2%) intermediate grade ductal carcinoma in situ (Solid pattern).
- * Additional findings: Tumor infiltrates & ulcerating the overlying skin.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Nipple & areola: Free.

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / ساميه محمود شمري على	Request No. : 13004076
Patient No. : 27307091701308	Request Date : 21/12/2025
Path No. : 3924/25	Verification Date : 01/01/2026
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- * Deep margin: Free.
- * Axillary lymph nodes & Extra-axillary lymph nodes: Seventeen lymph nodes free of viable metastatic deposits (0/17) (Multiple lymph nodes showing effect of therapy).
- * Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:

Residual viable invasive duct carcinoma, Grade 2 (Cellularity 70%) (Actual tumor size 3x3 cm).

- * Tumor infiltrates & ulcerating the overlying skin.
- * Ductal carcinoma in situ: Minimal (2%) intermediate grade ductal carcinoma in situ (Solid pattern).
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes & Extra-axillary lymph nodes: Seventeen lymph nodes free of viable metastatic deposits (0/17) (Multiple lymph nodes showing effect of therapy).
- * Stage: ypT4b, ypN0.
- * RCB: 2.735, RCB class-II.

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 05/02/2026 11:55:30

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

1231

Patient Name	: الأختلاف / سامية محمود غفرى على	Request No.	: 13004076
Patient No.	: 27307091701308	Request Date	: 21/12/2025
Path No.	: 3924/25	Verification Date	: 01/01/2026
Sex	: Female	Financial Category	: فئة المرضى - جويستاف روسي
Age	: 52 Y	Department	: Out Patient
Refereing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

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Gross Examination:

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- * Tumor ulcerating overlying skin.
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- * Additional findings: Tumor infiltrates & ulcerating the overlying skin.
- * Angiolinvasion: Absent.
- * Perineural invasion: Absent.
- * Nipple & areola: Free.

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Patient Name : الأستقاء / ساليه محمرد غبرى على
Patient No. : 27307091701308
Path No. : 3924/25
Sex : Female
Age : 52 Y
Refereing Doc :

Request No. : 13004076
Request Date : 21/12/2025
Verification Date : 01/01/2026
Financial Category : نفقة الأسرة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Deep margin: Free.
- Axillary lymph nodes & Extra-axillary lymph nodes: Seventeen lymph nodes free of viable metastatic deposits (0/17) (Multiple lymph nodes showing effect of therapy).
- Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:

Residual viable invasive duct carcinoma, Grade 2 (Cellularity 70%) (Actual tumor size 3x3 cm).

- Tumor infiltrates & ulcerating the overlying skin.
- Ductal carcinoma in situ: Minimal (2%) Intermediate grade ductal carcinoma in situ (Solid pattern).
- Nipple & areola: Free.
- Deep margin: Free.
- Axillary lymph nodes & Extra-axillary lymph nodes: Seventeen lymph nodes free of viable metastatic deposits (0/17) (Multiple lymph nodes showing effect of therapy).
- Stage: ypT4b, ypN0.
- RCB: 2.735, RCB class-II.

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 05/02/2026 11:55:30

Page 2 of 2

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Patient Name : الأستاذة / سامية محمود غمري على	Request No. : 13009700
Patient No. : 27307091701308	Request Date : 26/04/2025
Pg. No. : 1479/25	Verification Date : 08/05/2025
Sex : Female	Financial Category : ميسرة - ميسرة
Age : 51 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report



ERM036745592

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations. proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ghonim.

Dr. Samia Ali.

Verified by :

Dr.Toka Medhat

Printing Date : 13/05/2025 14:31:34

Page 2 of 2

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Patient Name : سامية محمود غنوي علي
Patient No : 1400-2025
Sex : Female Age : 52 Y
Financial Cate : حومتات زومبي - نفقة البرية
Department : Out Patient
Referring Doc :

Lab No. : 13C07066
Request Date : 06-09-2025 10:45
Verification Date : 06-09-2025 12:09
Sample Type : EDTA Blood
Test Status : Routine
Analyzers : XN350_ha.ml

Complete Blood Picture

Test	Result	Unit	Ref. Range
Hemoglobin	10.5	g/dL	12.0 - 16.0
RBCs	4.2	$10^6/\mu\text{L}$	4.0 - 6.5
Hematocrit (PCV)	31.7	%	36.0 - 50.0
MCV	75.1	fL	76.0 - 101.0
MCH	24.9	pg	26.0 - 32.0
MCHC	33.1	g/dL	31.5 - 34.5
RDW-CV	21.7	%	11.6 - 14.0
PLT	231	$10^3/\mu\text{L}$	150 - 450
WBCs.Total	4.1	$10^3/\mu\text{L}$	4.0 - 11.5

Differential Leucocyte Count :

	Relative		Absolute		
Neutrophils%	67.8	%	2.78	/mm	2.00 - 7.00
Lymphocytes%	30.3	%	1.24	/mm	1.50 - 3.50
Monocytes %	1.7	%	0.07	/mm	0.20 - 1.00
Eosinophils %	0.0	%	0.01	/mm	Up to 0.50
Basophils %	0.2	%	0.01	/mm	Up to 0.10

Signature

Dr./Radwa Younes

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Page 1 of 1

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25311121 / فاكس

25311126

ت / 25311125

987 فورنيش النيل العك الصانع

Patient No. : 1400-2025
Patient Name : الأستاذة / سامية محمود حمزى على
Sex : Female Age: 52 Y
Financial Cate: جوستاف روسي - نفقة الدولة
Department : Out Patient
Referring Doc :

Request No : 13007066
Request Date : 06-09-2025 10:45
Verification Date : 06-09-2025 12:11
Sample Type : Serum
Test Status : Routine
Analyzers : biolis_harmel

Clinical Chemistry Report

Test	Result	Unit	Ref. Range	
AST (SGOT)	40.5	U/l	Up to 31.0	)
Serum Creatinine	0.8	mg/dl	0.6 - 1.2	)
Urea	22.7	mg/dl	17.0 - 43.0	)
Bilirubin Total	0.46	mg/dl	0.10 - 1.20	)

Signature

Dr./Radwa Younes

Patient No. : 1400-2025
Patient Name : الأستاذة / أسماء محمود الخزرجي علي
Sex : Female Age : 52 Y
Financial Cat: جوفاء - رومسي - نفقة البررة
Department : Out Patient
Referring Doc :

Request No : 13002516
Request Date : 16-08-2025 10:29
Verification Date : 16-08-2025 11:28
Sample Type : Serum
Test Status : Routine
Analyzers : biolis_hermel

Clinical Chemistry Report

Test	Result	Unit	Ref. Range	
ALT (SGPT)	<u>38.1</u>	U/l	Up to 31.0	)
AST (SGOT)	<u>12.5</u>	U/l	Up to 31.0	)
Serum Creatinine	0.9	mg/dl	0.6 - 1.2	)
Uric acid	3.1	mg/dl	2.6 - 6.0	)
Urea	31.6	mg/dl	17.0 - 43.0	)
Bilirubin Total	0.38	mg/dl	0.10 - 1.20	)
Bilirubin Direct	0.10	mg/dl	Up to 0.20	)

Signature

Dr./Radwa Younes

Patient Name : سميرة محمود عمري علي
Patient No : 1400-2025
Sex : Female Age : 52 Y
Financial Code : بطاقة الدولة - حرمين
Department : Out Patient
Referring Doc :

Lab No. : 13002516
Request Date : 16-08-2025 10:29
Verification Date : 16-08-2025 10:56
Sample Type : EDTA Blood
Test Status : Routine
Analyzers : DXH_600_Hermel

Complete Blood Picture

Test	Result	Unit	Ref. Range
Hemoglobin	11.5	g/dL	12.0 - 16.0
RBCs	4.7	10 ⁶ /uL	4.0 - 6.5
Hematocrit (PCV)	34.6	%	36.0 - 50.0
MCV	74.2	fL	76.0 - 101.0
MCH	24.7	pg	26.0 - 32.0
MCHC	33.2	g/dL	31.5 - 34.5
RDW-CV	21.4	%	11.6 - 14.0
PLT	426	10 ³ /uL	150 - 450
WBCs Total	10.1	10 ³ /uL	4.0 - 11.5

Differential Leucocytic Count :

	Relative		Absolute		
Neutrophils%	70.2	%	7.09	/mm	2.00 - 7.00
Lymphocytes%	23.8	%	2.40	/mm	1.50 - 3.50
Monocytes %	5.5	%	0.56	/mm	0.20 - 1.00
Eosinophils %	0.0	%	0.00	/mm	Up to 0.50
Basophils %	0.4	%	0.04	/mm	Up to 0.10

Signature

Dr./Kadwa Younes

Printing Date : 16/08/2025 12:13:36

Page 1 of 1

Printing By : hermel

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- 25311125 / ت

987 عورنيش النيل، الملك الصالح



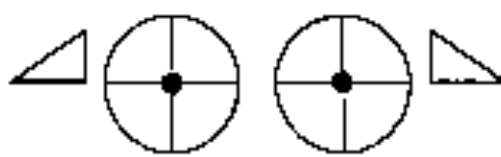
مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السرطان
مصر

MDT FORM 3 / 6 / 2025 G

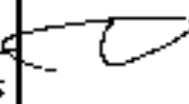
Name	سليمة محمود محمد	MRN	1900/2025
Age	51 yrs	ID	CV 2. V. 91 V. 12. A
Menopausal status	Post.	Phone	01221399003
NRS SCORE		Marital status	
BMI		Family history	no FH note
CO morbidity	Free	REASON OF MDT	

Clinical Data

Bx:	LC IDC II	R	L
ER:	8/8	PR:	5/8
HER 2:	3+	Ki67:	256
Sonomammography:	LC locally advanced malignancy		
Mets Workup:	PET-CT → low grade metastatic disease. Small LC Tumor. LC subcutaneous metastases.		

Recommendations:	
MDT OUTCOME & PLAN	for TC+AP X 6 cycles then MRM.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
	Dr. Hani 3/6/2025				

87

Patient Name	: الأستاذة / سامية محمود شعري علي	Request No	: 13009700
Patient No.	: 27307031701308	Request Date	: 26/04/2025
Path No.	: 1479/25	Verification Date	: 08/05/2025
Sex	: Female	Financial Category	: فور لم دار الصلعم - مبللرم
Age	: 51 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 26/4/2025.

Gross Examination:

- Multiple gray and white tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immunostaining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis). (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal DCIS (comedo type).
- * Angiolysis: Undetected.
- * Perineural invasion: Undetected.
- * Microcalcifications: Undetected.
- * IPT: (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 5% of tumor cells (score 5/8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 25% nuclear reaction of tumor cells.

Diagnosis.

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

Verified by :

Dr.Toka Medhat

د. طارق مهن

م. محمد

Printing Date : 11/05/2025 14:31:34

Page 1 of 2

Printing By : marwa mohamed youss



PATIENT NAME: SAMIA MAHMOUD Ghamry A_I

GENDER: Female

DATE : 15-May-2025 09:47 AM

PATIENT ID: 10125263324

AGE: 51 y

ORDER: 3379473

WHOLE BODY FDG PET-CT EXAMINATION

Clinical background:

- 51-year-old female patient.
- Presented with left breast pathologically proven IDC grade I.
- Weight = 86 Kg.
- BL glucose level = 111 mg/dL.

Reason of study:

- Initial staging study.

Radiopharmaceutical:

- F-18-FDG 5.8 mCi injected.

Technique:

- Approximately one hour after IV injection of the radiopharmaceutical, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- **Reference hepatic activity (SUV max)~ 2.24.**

Findings:

Reviewing PET, CT and fused PET-CT images:-

1- Breasts and axillary regions:

- Left breast UOQ metabolically active (FDG avid) soft tissue mass is seen infiltrating the overlying skin, measuring about 7.8 x 12.2 cm with SUV max~19.58.
- Multiple FDG avid pathologically enlarged left axillary lymph nodes (levels I, II and III), the largest measures about 4.2 x 5 cm with SUV max~16.55 at level I.

PATIENT: SAMIA MAHMOUD Ghany ALI. 1C125263324, Female, 51 y

Report version 5, 17-May-2025 05:40 PM

Page 1 of 3



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10612



هذه هي الصورة التي تظهرها الوثائق التي تم العثور عليها في الجدران.



هذه المصنفات هي من المصنفات التي
 في علم الفقه الإسلامي

- **Few low grade metabolically active left internal mammary lymph nodes, the largest measuring about 0.8 cm with SUV max about 1.31.**
- **Few prominent low grade metabolically active left supraclavicular lymph nodes, the largest is measuring about 0.8 x 1.5 cm with SUV max about 1.24.**
- No metabolically active (FDG avid) contralateral breast masses or axillary lymph nodes.

II. Head and neck:

- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aero-digestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and ary-epiglottic folds. No glottic, supra- or infraglottic masses.
- Intact laryngeal skeleton.
- No FDG avid thyroid nodules.

III. CHEST:

- No metabolically active (FDG avid) pulmonary nodules.
- Non FDG avid right upper lung lobe atelectatic bands likely of old granulomatous disease.
- **Few low grade metabolically active mediastinal (right para tracheal, prevascular, retrocaval and subcarinal) lymph nodes, the largest at subcarinal group measuring about 1.3 x 1.7 cm with SUVmax about 2.87.**
- No metabolically active pleural effusion or masses.
- Average cardiac size. No pericardial effusion detected.

IV. ABDOMEN AND PELVIS:

- The liver is enlarged with homogenous low CT texture (fatty) with segment IV isometabolic area of relative hyperdensity mostly representing focall fatty sparing area. No definite CT evidence of metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal appearance of the pancreas, spleen, both kidneys and both suprarenals (on non contrast CT basis) with No FDG avid focal lesions.
- No metabolically active abdominal or pelvic lymphadenopathy.
- Under filling of the urinary bladder with no filling defects or diverticular outpouching.

PATIENT: SAMIA MAHMOUD Ghemry ALI, 10425203324, Female, 51 y

Report version 5, 17-May-2025 05:40 PM

Page 2 of 3



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16602

مركز صحة اليوم
4 شارع ملوحة قبل شارع البردنج طابق الطابق

بمركزنا في إزوية الشيخ زايد
د. علي عثمان - الشيخ زيد محمد طه - د. دة

- No ascites.

V. Scanned axial and proximal appendicular skeleton:

- No metabolically active (FDG avid) osseous lesions.

OPINION:

A reported case of left breast cancer, at initial staging showing:

- Left breast metabolically active (FDG avid) mass lesion with metabolically active (FDG avid) ipsilateral pathological axillary lymphadenopathy.
- Low grade metabolically active small left internal mammary, left supraclavicular and mediastinal lymph nodes for close follow up.
- No definite metabolically active (FDG avid) pulmonary, hepatic or osseous metastatic deposits.

Reported By,

Dr. Fifi Samy, MSc

Dr. Mohamed Mohsen, MD

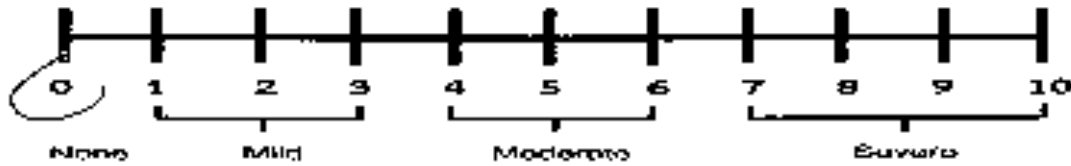
Prof. Huda Fathy, Nuclear Medicine Consultant

17-May-2025 05:40 PM



اسم المريض ثلاثي:
رقم الملف:

تقييم الألم:



التاريخ	الوقت	درجة الألم	مكان الألم	نوع الألم	العلاج	توقيع التمريض
١٨/٨						

*العلامات الحيوية

الوقت	ضغط الدم	النبض	الحرارة	معدل التنفس	نسبة تشبع الدم بالأكسجين	سكر عشوائي بالدم	الوصلات درجة الوعي

*خطة الرعاية التمريضية:-

احتياجات المريض	الاجراءات	النتائج المرجوة	الانطار الزمني	توقيع التمريض

*الانوية والمحليل:-

توقيع التمريض	توقيع الطبيب	شريقة الأعطاء	التركيز	الجرعة	العلاج كاو الدواء	الوقت
الوقت	ملاحظات التمريض					التوقيع

اسم المريض ثلاثي:

رقم الملف:

short stay نموذج تقييم ومتابعة مريض الإقامة القصيرة للكبار

تَحْلِيلِيَّةٌ :

نعم () لا ()
 (في حالة نعم) : تولى عليه (في حالة لا) : (.....)

البرك
الزوجة
طول

أو ع... (13) (المرء لثمة و'عرو اغلا و'عروا

هل هناك منقذات ثلاث مع تعرضي؟

☒ لا أظنني بحاجة نعم ☐ نعم ☐ هل تعلم مع المريض ☐ هذا بواسطة |

* لکھنؤ، ۲۰ ستمبر

II سرورۃ التعريف II عدم التفسير II مخطوطات القمي II المخطوطات - اسطرط

تفہیم

1- لمعني اللد : [جاءه] (0) [وقب] (1) [عصا] (2)

2- التدرج المرضي: [طبي (غشبي) (1)] [صحة في البلع (2)] [انخفاض الوزن حول (3)]

7. تنفيذ برامج (3) [تنفيذ بالمسؤول الجديدة] (4)

3. تنبيه : [يعني (1)] [الكل من المعطلة (2)] [ملغود (2)]

4- الأنثى : ☐ مقام اسنان (7) ☐ لا توجد اسنان مطرودة (D) ☐ (كالحسن مطرودة) (1)

١٠ - مريض الفيلوخته اكثر من ٧٥ سنة لظواهر (اجرو) و محنة مزاجية (١)

سبعة أحمل - رضاعة ١١١

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.....

$\frac{1}{2} \times \frac{1}{2} \times \frac{1}{2}$
 $\frac{1}{2} \times \frac{1}{2} \times \frac{1}{2}$

ع	ع	ع	ع	ع
ع	ع	ع	ع	ع
مكان القردة	المجموع الفرعية	الذين	الإجراءات	

[illegible]

ECHO REPORT

Name:

Dr. Mina Adel

Date: 23 / 6 / 2025

EF	66%	55-75%	PWT	0.8	0.6-1.1cm
LV EDD	4.3	3.5-5.8cm	AO.R	2.3	2-3.7cm
LVESD		2-4cm	LA	3.8	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Ao root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

Dr / Mina Adel

(Signature)



Patient No. : 27307091701308
Patient Name : الأستاذة / ماريه محمود غمري عني
Sex : Female Age : 51 Y
Financial Cate : أورام - دار السلام - مبنية
Department : Out Patient
Referring Doc :

Request No : 13002424
Request Date : 23-06-2025 09:40
Verification Date : 23-06-2025 11:43
Sample Type : Serum
Test Status : Routine
Analyzers : bioss_hemel

Clinical Chemistry Report

Test	Result	Unit	Ref. Range	
ALT (SGPT)	<u>44.7</u>	U/l	Up to 31.0	)
AST (SGOT)	<u>46.2</u>	U/l	Up to 31.0	)
Serum Creatinine	1.0	mg/dl	0.6 - 1.2	)
Urea	37.0	mg/dl	17.0 - 43.0	)
Bilirubin Total	0.34	mg/dl	0.10 - 1.20	)
Bilirubin Direct	0.07	mg/dl	Up to 0.20	)

Signature

Dr.Sahar Mansour

Printing Date : 23/06/2025 13:34:27

Page 1 of 1

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987 كورتيشن النيل الملك انصالح



Patient Name : سمية محمود غمري عني
Patient No : 27307091701308
Sex : Female Age : 51 Y
Financial Cat : أورام ناز السلام - مبالغ
Department : Out Patient
Referring Doc :

Lab No. : 13002424
Request Date : 23-06-2025 09:40
Verification Date : 23-06-2025 10:43
Sample Type : EDTA Blood
Test Status : Routine
Analyzers : XN350_hermi

Complete Blood Picture

Test	Result	Unit	Ref. Range
Hemoglobin	11.5	g/dL	12.0 - 16.0
RBCs	4.6	10 ⁶ /uL	4.0 - 6.5
Hematocrit (PCV)	35.3	%	36.0 - 50.0
MCV	77.6	fL	76.0 - 101.0
MCH	25.3	pg	26.0 - 32.0
MCHC	32.6	g/dL	31.5 - 34.5
RDW-CV	14.9	%	11.6 - 14.0
PLT	321	10 ³ /uL	150 - 450
WBCs.Total	6.4	10 ³ /uL	4.0 - 11.5

Differential Leucocytic Count :

	Relative		Absolute		
Neutrophils%	50.1	%	3.21	/cmm	2.00 - 7.00
Lymphocytes%	40.1	%	2.57	/cmm	1.50 - 3.50
Monocytes %	7.5	%	0.48	/cmm	0.20 - 1.00
Eosinophils %	2.0	%	0.13	/cmm	Up to 0.50
Basophils %	0.3	%	0.02	/cmm	Up to 0.10

Signature

Dr.Sahar Mansour

Printing Date : 23/06/2025 13:34:29

Page 1 of 1

Printing By :hermal

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987 كورنيش النيل العاك الصالح



Patient Name: Samia Mahmoud Gamri Al.	Date : 22/04/2025
Exam : MAMMOGRAPHY BOTH	Age : 51 Year(s)
Medical ID: 27307091701308	Pat. ID: 99094

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Report:

- ACR b: scattered fibroglandular parenchyma
- Left Breast:
 - A large dense mass is seen in the UOQ with overlying skin infiltration measuring about 102x60x100mm.
 - Diffuse edema pattern likely due to lymphedema
 - Multiple pathologically enlarged levels I , II & III axillary lymph nodes are noted showing thick cortices and effaced hila. The largest nodes are forming a huge nodal mass measuring about 43x28mm.
 - No supraclavicular lymphadenopathy.
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Locally advanced left breast malignant mass and multilevel axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University

22/04/2025 4:30 PM



الوقت	درجة الألم	مكان الألم	نوع الألم	العلاج	توقيع التمريض
١٢:٣٠	٢				حسينة العيسى

*العلامات الحيوية

الوقت	ضغط الدم	النبض	الحرارة	معدل التنفس	نسبة تشبع الدم بالأكسجين	معدل عشوائي بالألم	الوصلات	درجة الوعي
١٢:٣٠	٩٠/٦٠	٩٠	٣٦	٢٢	١٥٥		سليمة واعي	

*خطة الرعاية التمريضية:-

احتياجات المريض	الإجراءات	التدخلات المرجوة	الانذار الزمني	توقيع التمريض

*الادوية والمحاليل:-

توقيع التمريض	توقيع الطبيب	طريقة الأخطاء	التركيز	الجرعة	العلاج أو الدواء	الوقت
		١٠٧			سليمة	١٢:٣٠
					السليمة	١٢:٣٠
					سليمة	١٢:٣٠
الوقت	ملاحظات التمريض					التوقيع
١٢:٣٠	تم تركيب سليمة واعي					حسينة العيسى
١٢:٣٠	تم بدء العلاج					حسينة العيسى
	تم إعطاء العلاج					حسينة العيسى

مطلوبہ: مطلوبہ

٧٠ هـ: تلك ملتهبات كتبت مع العرب بن؟

وہاں پہنچ کر اس نے

تلاويح الخفية

- درجة تليوم النخلة : مدهالي في طبيعي بلار للتليم اعلى من 4 يتم هدا في طبيب و
نوع النخلة المشعة بالمعزول : 3

[illegible]

في اخرى من الامثلة

فِيهِ تِلْكَ الْفَلَاةُ صَبَا

د. أحمد كمال الدري
د. محمد أبو الهدي درويش
د. عمر حسين عمر
د. وحيد حسين طنطاوي
د. ياسر عبد العظيم عباس
د. أحمد محمد منيب
أساتذة الأشعة التشخيصية
كلية طب - جامعة عين شمس

Patient Name: Hanan Adhy Mohamed	Study Date: 30/04/2025
Ref. Phy.: Dear Sir	Patient ID: 1294086

BILATERAL 3D DIGITAL BREAST MAMMOGRAPHY WITH HIGH RESOLUTION ULTRASOUND, REVEALED:-

FINDINGS:-

- Right breast outer quadrant an ill defined opacity, is irregular shaped, isodense, with partial speculated like margins.
- Both breasts show normal nipple, areola, skin & subcutaneous tissue thickness.
- Both breasts show normal distribution of fatty & diffuse scattered dense glandular element (ACR-B).
- No left side suspicious masses or micro calcifications.
- Bilateral axillary mild enlarged lymph nodes.

High resolution ultrasonography:

- Right breast between 7 & 8 O'clock 6 cm from nipple shows an irregular shaped, markedly hypoechoic, speculated soft tissue lesion (18 x 13 x 11 in width, depth, & CC).
- Bilateral breast central ductectasia up to 5 mm with clear fluid contents.
- Both breasts show normal rest of skin, subcutaneous tissue, nipple and areola.
- No left solid or cystic mass.
- No suspicious enlarged axillary lymphadenopathy in both sides.

IMPRESSION:-

- Right breast suspicious mass lesion BI RADS 4c, for biopsy
- Bilateral central ductectasia BI RADS 2.

MUCH OBLIGED

Dr.Nadin Fawzy.MD

ا.د. أحمد كمال الدري
 ا.د. محمد أبو الهدى درويش
 ا.د. عمر حسين عمر
 ا.د. وحيد حسين ططاوي
 ا.د. جاسر عبد العظيم عباس
 ا.د. أحمد محمد منيب
 أساتذة الأشعة التشخيصية
 كلية الطب – جامعة عين شمس

Patient Name: Hanan Adly Mohamed	Study Date: 30/04/2025
Ref. Phy.: Dear Sir	Patient ID: 1294086

BI-RADS (Breast Imaging Reporting and Data System):

BI-RADS 0	Need additional imaging evaluation and/or prior mammograms for comparison.
BI-RADS 1	Negative
BI-RADS 2	Benign finding
BI-RADS 3	Probably benign finding Initially: Short interval follow-up is suggested Less than 2% risk of malignancy.
BI-RADS 4	Suspicious abnormality Biopsy should be considered, wide range of probability of malignancy.
BI-RADS 4 Category 4a	Low suspicious for malignancy.(2-10%)
BI-RADS 4 Category 4b	Moderate suspicious for malignancy.(10-50%)
BI-RADS 4 Category 4c	High suspicious for malignancy (50-95%)
BI-RADS 5	Highly suggestive of Malignancy. Appropriate action should be taken, with a 95% likelihood of malignancy.
BI-RADS 6	Known biopsy proven malignancy.

ACR classification of breast density:

- ACR-a: Almost entirely fatty.
- ACR-b: Scattered areas of fibro glandular density.
- ACR-c: Heterogeneously dense.
- ACR-d: Extremely dense

مع مراجعة العقالين
OK

TCMP

Adjust 66kg
(C3)

ايفيل + ديكال + ميراثريك + كونترولست + نيل الكيارك
ناكسوتير (140) حجم

AUCS كاربولانين

(C2) صبيو 70 / 70 قست الجله

كونترولست 1x1

ميراثريك 1x1

ميتابروفين 1x1

بارامول 2x1

ميتلنكس 1x1

هيبيركليك 1x1

Alpha Hanc Rul
C3TCMP = C2Pango
1618

م.ف
سيرة ذاتية
c. 18/17
ع حروف فسيو ناكسوتير
+ كاربولانين + ناكسوتير

TCMP

C4 TC C3 HP

ايفيل 2x1 ديكال + 2 كونترولست + 2 ميراثريك
ناكسوتير 140 حجم

AUCS كاربولانين

صبيو 70 / 70 قست الجله

كونترولست 1x1x1x1

ميراثريك 5x1x1

ميتابروفين 5x1x1

ميتلنكس 1x1x1x1

بارامول 1x1x1x1

هيبيركليك 2x1x1x1

بارامول 1x1x1x1

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labs pending
Echo (R)

ع.ف
49

Clinical pharmacy
C4 TC
G HP

Signature
c. 19/17

ر.ف
c. 19/17

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى أوزام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

C-50 / 71CA

Clinical findings:

Sl. No. 1 Pt. with H. breast
Cancer. IAG Grade 2 ER 8/8 PR 5/8
Hx 2 + 3 L4/5 7 25/1
With +ve family history father

	Doctor Order	الوقت
EF: %	- for TCHP 16	Echo → EF 66%
BP:		23/6/2025
TEMP:	C, TCHP	WT = 86
HR:	أول 10 ديسا + جاستريك + كونترولول	Ht = 158
HB:		PSA = 1.87
WBC:	بالطال	120 كبحومير
PLT:	PLC = 6.4	كاربولاسيت 0.6
UREA:	HT = 32/	قلبي 1200 / 600 تحت الحلة (البريوت)
CREAT:	creat = 1	
ALT:		كونترولول
AST:		ايكسا
T.BIL:		سيروكيت
D.BIL:		لداغول

Clinical pharmacy
C, TAT / Cab
(65mg)
C-50 / 71CA

Doctor Signature:

Date: / / 20

Time:

اسم المريض ثلاثي:
رقم الملف:



وزارة الصحة و السكان
أمانة المراكز الطبية المتخصصة
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PHYSICIAN ASSESSMENT SHEET

I. PERSONAL HISTORY:-

Occupation:
Marital Status : ☐ Married ☐ Single ☐ Widow ☐ Divorced
Special Habits : ☐ Smoking ☐ Coffee ☐ Alcohol ☐ Others
Allergy: ☐ No ☐ Yes - Food: Medications: Others: ☐ Unknown

II. HISTORY:

1.Past History: - ☐ DM ☐ HTN ☐ CLD ☐ IHD ☐ CRD ☐ CVS

☐ Others, specify:
☐ Previous Operations, specify:
☐ Previous Admission to Hospital, ☐ No ☐ Yes Comment

2.Family History:

.....
.....
.....
• Menstrual history : Age at Menarche : / Y Ferquency : / Days .
• Regular : No / Yes Duration : / Days Pain: No / Yes Discharge: No /
• Menopause: No / Yes Duration : / Y
• History of location:
• History of Miscarriage still birth : No / Yes (Times)
• Children: No. : Sex: Age:

3.Psychological History (Including Emotional/ Behavior/ Social Factors)

.....
.....
.....
.....
.....
.....

4.Current Medication:

Drug	Strength	Dosage/ Frequency	ROUTE
.....			
.....			
.....			

5.Date of First Diagnosis of Cancer

/ / 20

6.Date of First Presentation in Our Unit

/ / 20

7.Present History:

A. The Condition Started by:

.....
.....
.....

اسم المريض ثلاثي:
رقم الملف:/.....



وزارة الصحة و السكان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام " هرمل "

Oncology Follow up Sheet

صحيفة متابعة مريض اورام (☐ اطفال / ☐ كبار)

اسم المريض:	
الرقم الطبي:	رقم الملف:
المعاملة المالية:	الرقم القومي:
الطبيب المعالج:	
العنوان الدائم:	
رقم التليفون:	الجنسية:
النوع:	العن:
تاريخ الميلاد:	تاريخ اول زيارة:
كود الأشعة:	